@apaji_x ADHD claims: evidence check (12 Oct 2026)

Date checked: 12 Oct 2026, 8:06 AM IST

Scope: posts from 11 Oct 2026, 9:39 AM IST to 12 Oct 2026, 3:40 AM IST (75 original posts in 12 self-reply threads; 49 reposts not checked)

3 Supported	6 Partially supported	5 Insufficient evidence	5 Unsupported

Section 1 · Claims

Supported 3

Claim #C1 · Supported · High confidence

Research Quality: Claim: anxiety and ADHD often occur together. A systematic review of 32 studies (Level 1, moderate consensus) finds anxiety disorders among the most common comorbidities in adults with ADHD, more common than in adults without ADHD [1]. GRADE moderate: consistent observational data, but no pooled estimate and diagnostic tools vary. Larger studies using a single structured interview would sharpen the numbers.

X Post Link · Citations for this claim

Anxiety and ADHD often show up together, and food can quietly make it better or worse. Here is how to eat in ways that keep your body calmer and your mind steadier. 1/6

1/6 – Anxiety and ADHD often show up together, and food can quietly make it better or worse. Here is how to eat in ways that keep your body calmer and your mind steadier. 1/6

2/6 – Skipped meals, too much caffeine and poor sleep can all mimic or amplify anxiety. A racing heart, shaky hands and a restless mind may be your body asking for fuel, not just stress. 2/6

3/6 – Fix 1: Eat regularly. Long gaps between meals can leave you jittery and on edge. Something with protein every 3 to 4 hours helps your body feel more stable. 3/6

4/6 – Fix 2: Go easy on caffeine. Coffee, energy drinks and strong tea can turn up the jitters, especially on an empty stomach. Try smaller cups, eat first and set an early afternoon cutoff. 4/6

5/6 – Fix 3: Add calming basics. Oats, bananas, leafy greens, nuts, seeds and yogurt are everyday foods that fit into a steady, balanced eating pattern. Think of them as support, not a cure. 5/6

6/6 – Fix 4: Pair food with a pause. Take five slow breaths before you eat or step outside for a minute after. A small reset helps your body settle and makes meals feel less rushed. 6/6

Claim #C2 · Supported · Moderate confidence

Research Quality: Claim: too much caffeine and poor sleep can amplify anxiety. Blinded placebo-controlled caffeine challenges raise anxiety in healthy adults and trigger panic attacks in about half of people with panic

disorder [1], and experimental sleep loss raises next-day anxiety [2] (GRADE moderate). Red flags: caffeine doses were high (about 5 cups of coffee), samples were small, and neither study involved ADHD. The "skipped meals mimic anxiety" part is covered under C12.

X Post Link · Citations for this claim

Anxiety and ADHD often show up together, and food can quietly make it better or worse. Here is how to eat in ways that keep your body calmer and your mind steadier. 1/6

Same post as Claim #C1 ↑

Claim #C3 · Supported · High confidence

Research Quality: Claim: stimulant medication can suppress appetite for hours. The Cochrane review of methylphenidate RCTs names decreased appetite as a typical non-serious adverse effect [1] (Level 1, strong consensus). The review rates overall adverse-event certainty very low because trials are hard to keep blinded; we rate this specific, consistently reported effect moderate. The thread's advice to ask the prescriber about meal timing is sound.

X Post Link · Citations for this claim

Does when you eat matter as much as what you eat? For ADHD brains, regular meal timing may be one of the simplest levers for steadier focus. Here is what is known, and what is still unclear. 1/7

1/7 – Does when you eat matter as much as what you eat? For ADHD brains, regular meal timing may be one of the simplest levers for steadier focus. Here is what is known, and what is still unclear. 1/7

2/7 – Research on meal timing and attention in ADHD is limited. What we do know comes from broader work: going long stretches without food can affect blood sugar, mood and concentration in many people, and ADHD can make those effects easier to notice. 2/7

3/7 – ADHD adds a practical problem on top of the biology. Hunger cues can be quiet during hyperfocus, and stimulant medication can suppress appetite for hours. Many people end up eating little all day and a lot at night. 3/7

4/7 – This pattern has a name in everyday use, front-loading versus back-loading calories. It is not a formal diagnosis. It simply describes eating most of your food late in the day, often after medication wears off. 4/7

5/7 – A steady approach is to aim for something every 3 to 4 hours, even if small. Breakfast with protein, a planned midday meal and a mid-afternoon snack give your brain regular fuel instead of one big catch-up at night. 5/7

6/7 – If you take stimulant medication, a small protein-rich meal before the dose wears in, or a calorie-dense snack during the day, can help. Ask your prescriber what timing works for you, since advice differs by medication. 6/7

7/7 – Simple tools help more than willpower. Phone alarms labeled EAT, snacks within arm's reach and one repeatable breakfast remove the need to remember or decide when your brain is busy. 7/7

Partially supported 6

Claim #C4 · Partially supported · Moderate confidence

Research Quality: Claim: regular meals may steady focus in ADHD, and many people with ADHD eat little by day and a lot at night. The post itself correctly says direct ADHD research is limited. Breakfast beats fasting for same-morning attention in children (systematic review, Kellogg's-funded) [1], and ADHD carries a roughly fourfold risk of binge-eating disorder [2] (GRADE low). Red flags: indirect populations and industry funding; a trial of meal timing on attention in adults with ADHD would strengthen it.

X Post Link · Citations for this claim

Does when you eat matter as much as what you eat? For ADHD brains, regular meal timing may be one of the simplest levers for steadier focus. Here is what is known, and what is still unclear. 1/7

Same post as Claim #C3 ↑

Claim #C5 · Partially supported · Moderate confidence

Research Quality: Claim: ADHD "time blindness" makes people underestimate durations, lose track of time and run late. Two meta-analyses (Level 1) find real timing deficits, g≈0.4–0.7 [1] [2], and planning and set-shifting weaknesses fit the "hard to switch" sign [3] (GRADE moderate for the deficit). However, these are lab timing tasks; no study links them to the listed everyday signs or to lateness, and "a psychiatrist explains" names no source. Real-world time-management studies would strengthen it.

X Post Link · Citations for this claim

A psychiatrist explains why your “always running late” habit may sometimes be ADHD-related time blindness.

1/7 – A psychiatrist explains why your “always running late” habit may sometimes be ADHD-related time blindness.

Here are the signs: 1/7

2/7 – 1. You think everything will take five minutes.

Getting dressed, making breakfast, or replying to one message seems quick.

But somehow, a 15-minute task turns into 45 minutes before you realize it. 2/7

3/7 – 2. You lose track of time while doing something interesting.

You start watching one video or working on one project.

You look up, and suddenly an hour has disappeared without you noticing. 3/7

4/7 – 3. You underestimate how long getting ready takes.

You calculate the time needed to reach your destination but forget about showering, finding your keys, choosing clothes, and getting out the door.

You end up rushing even when you started preparing early. 4/7

5/7 – 4. You keep saying, “I'll do it in a minute.”

You finish one small thing before leaving.

Then you check one message, put something away, or start another task.

That minute keeps stretching until you're late. 5/7

6/7 – 5. Deadlines feel unreal until they're close.

A task due next week doesn't feel urgent today.

Then suddenly, the deadline is tomorrow, and you have to complete everything at once. 6/7

7/7 – 6. You struggle to switch from one activity to another.

You know you need to leave, but stopping what you're doing feels difficult.

You keep thinking, “Just one more thing,” until there's barely enough time. 7/7

Claim #C6 · **Partially supported** · Moderate confidence

Research Quality: Claim: "high-functioning" adults with ADHD hide real impairment behind private systems and effort. A small case-control study found adults with ADHD and higher IQ showed fewer executive-function deficits on tests, suggesting compensation that can mask diagnosis [1], and patients describe many self-built strategies [2] (GRADE low). Red flags: "high-functioning ADHD" is not a diagnosis, samples are small or qualitative, and the 7 specific signs are untested. A larger study of compensation and its cost would strengthen it.

X Post Link · Citations for this claim

7 SIGNS YOU HAVE HIGH-FUNCTIONING ADHD that nobody in your life has ever noticed.

1/6 – 7 SIGNS YOU HAVE HIGH-FUNCTIONING ADHD that nobody in your life has ever noticed.

And why everyone thinks you're fine. 1/6

2/6 – 1. You are always the last one to leave and the last one to start. You get things done. On time. Sometimes impressively so. But the cost of what it took to get there is completely invisible. The output looks normal. The process was a private emergency. 2/6

3/6 – 2. You function on interest and urgency only. The things you care about get done brilliantly. The things you do not care about quietly pile up until they become a crisis. From the outside this looks like selective effort. From the inside it is the only mode available. 3/6

4/6 – 3. You are exhausted in a way that cannot be explained by what you actually did. You had a quiet day. One meeting. Lunch with a friend. You are destroyed. Because what the day looked like and what the day cost your nervous system are two completely different things. 4/6

5/6 – 4. You have a system for everything and the systems are invisible and if someone disrupts one of them your whole day falls apart in a way that seems disproportionate. The system is not rigidity. The system is prosthetic executive function. Remove the prosthetic and you fall. 5/6

6/6 – 5. You got good grades. You hold down a job. You seem fine in public. You seem like you have it together. And you have spent so long making that true from the outside that almost nobody has ever seen what it takes to keep it there.

High functioning does not mean it is not hard. It means the effort is hidden. 6/6

Claim #C7 · **Partially supported** · Moderate confidence

Repeat of [11 Oct 2026 Claim #9 – Partially supported (GRADE Low)]
New in this wording: Adds that crisis adrenaline raises dopamine and that crises need ever bigger stimulus. Novelty does drive the dopamine midbrain [1], but high stress impairs prefrontal function [2], and the escalation claim is untested. Same verdict as claim 9.

X Post Link · X Post Link 2 · Citations for this claim

BREAKING: Neuroscientists confirm ADHD brains don't have a motivation problem.

1/6 – BREAKING: Neuroscientists confirm ADHD brains don't have a motivation problem.

They have a dopamine access problem.

Motivation responds to discipline.
Dopamine responds to urgency, reward, novelty.

You've been solving the wrong problem your entire life. 1/6

2/6 – 1. Discipline assumes you can choose to engage the motivational system at will. That is true for most people. It is not true for ADHD. The motivational system in ADHD is not voluntary. It fires when dopamine is available. It does not fire when dopamine is not available. Choosing has nothing to do with it. 2/6

3/6 – 2. This means every time you tried harder and it still did not work, you were not weak. You were applying a solution to a problem it was never designed to solve. Discipline is the right tool for a motivation problem. It is the wrong tool entirely for a dopamine access problem. 3/6

4/6 – 3. Urgency works because it produces adrenaline which raises dopamine temporarily. Reward works because anticipating a positive outcome activates the dopamine system. Novelty works because new things trigger dopamine release automatically. None of these are discipline. All of them are chemistry. 4/6

5/6 – 4. The cruelest part of this is that the people telling you to try harder are not wrong about what works for them. Discipline does regulate motivation in a neurotypical brain. They are giving you the correct answer to a different question. The question you are asking has a different answer. 5/6

6/6 – 5. Stop trying to motivate yourself into dopamine. Start engineering your environment to produce the dopamine your brain needs to function. That is not cheating. That is the correct solution to the actual problem. 6/6

ALSO CITED: BREAKING: SCIENTISTS CONFIRM THAT ADHD BRAINS PHYSICALLY CANNOT START A TASK WITHOUT DOPAMINE ACCESS.

1/6 – BREAKING: Scientists confirm that ADHD brains physically cannot start a task without dopamine access.

This is why "just do it" has never once worked.
This is why the deadline two weeks away doesn't exist.
This is why the crisis 1 hour away suddenly unlocks everything.

You're not lazy. You're running on the wrong fuel system. 1/6

2/6 – 1. The initiation system in the ADHD brain is not wired to respond to importance or intention. It responds to dopamine. If the dopamine is not there, the task does not start. It is that simple and that brutal. You can want it with your whole being and still sit there unable to move. 2/6

3/6 – 2. The two week deadline does not exist because it lives in not now. The future in ADHD is not a continuous line. It is a void. Things in the void are not real. The project due in two weeks is somewhere in the void and it will stay there until urgency drags it into now. 3/6

4/6 – 3. The crisis unlocks everything because crisis produces adrenaline. Adrenaline raises dopamine temporarily. The brain finally has the fuel it needed and it performs. Sometimes brilliantly. The performance is real. The cost of getting there through crisis every single time is also real. 4/6

5/6 – 4. This pattern works until it does not. The adrenaline system runs down. The crises start requiring bigger and bigger stimulus to produce the same effect. The person who used to do everything at the last minute starts finding that even the last minute is not enough anymore. 5/6

6/6 – 5. The goal is not to eliminate urgency as a fuel. It is to learn how to manufacture it earlier and more cheaply. Fake deadlines with real consequences. Body doubling. Timed challenges. External accountability. All of these produce dopamine without requiring a genuine emergency. 6/6

Claim #C8 · **Partially supported** · Moderate confidence

Research Quality: Claim: tying each new habit to an existing one beats trying a whole new routine. If-then planning has a medium-to-large effect on goal attainment (meta-analysis of 94 tests) [1], it helped children with ADHD on a lab inhibition task [2], and habits form through repeated cues [3] (GRADE moderate in general, low for ADHD). Red flags: older syntheses by the method's originators and no ADHD trial of routines. The other morning tips are covered under C13.

X Post Link · Citations for this claim

WAKE UP EARLY. JOURNAL. MEDITATE. WHOEVER WROTE THAT MORNING ROUTINE NEVER MET AN ADHD BRAIN.

1/7 – WAKE UP EARLY. JOURNAL. MEDITATE. WHOEVER WROTE THAT MORNING ROUTINE NEVER MET AN ADHD BRAIN.

You lasted four days. Then you decided the problem was you.
The ADHD-friendly version that actually works looks nothing like it. 1/7

2/7 – 1. The first thing you reach for runs your whole morning.

ADHD mornings run on dopamine, the brain's "let's go" chemical. Grab the phone, and TikTok gives it to you. Forty minutes later, you're still in bed.
Music, a stretch, a podcast you love. Your brain still gets it, and you're already up.
Win those first minutes, and you walk into work ahead of everyone still scrolling. 2/7

3/7 – 2. Forget the perfect morning. Go for a started one.

The perfect routine has five steps. ADHD paralysis stops you at the first one.
Make step one almost too easy. Sleep in your workout clothes. Put the phone across the room, so stopping the alarm means standing up.
Nobody at work sees how messy the start was. They see that you showed up. 3/7

4/7 – 3. Skip the full routine. Hang each new habit on an old one.

A brand-new routine is ten things to remember. ADHD working memory loses them by Wednesday.
So tie every new step to something you already do.
"I pour coffee → I fill my water bottle."
"I brush my teeth → I put on SPF."
Small wins add up. That's the feeling "disciplined" people keep bragging about. 4/7

5/7 – 4. Make breakfast boring on purpose.

Cooking at 7 am means too many choices, too early. So you skip it.
Protein shake. Yogurt cup. Literally anything.
Fuel your brain before the 10 am meeting, not halfway through it. That's the meeting where your boss turns to you first. 5/7

6/7 – 5. Hear the time, since you can't see it.

ADHD time blindness turns "five more minutes" into forty. The clock is right there. It just doesn't register.
Make a playlist with one song per step. Song one, out of bed. Song three, shower. Last song, keys in hand.
Skip it, and you're sending the "running late" text again. Third one this month. Somebody's counting. 6/7

7/7 – Go back to day five of the old routine.
Alarm at five. Journal still blank. Meditation app never opened. The "running late" text, sent anyway.
You counted that as proof you can't stick to anything.
Now count what it asked of you before seven. Three new habits at once. Nothing your brain wanted. No

clock you could hear.
Yes, you quit. You quit a routine built for someone else's brain. 7/7

Claim #C9 · Partially supported · Moderate confidence

Research Quality: Claim: in ADHD a deadline two weeks away feels unreal until urgency makes it "now". Steeper discounting of delayed rewards in ADHD is supported by a meta-analysis of case-control studies [1], and timing deficits add to it [2] (GRADE low). "The future is a void" overstates a moderate group difference. Studies of real deadlines and task start-up in adults with ADHD would strengthen it.

X Post Link · Citations for this claim

BREAKING: Scientists confirm that ADHD brains physically cannot start a task without dopamine access.

Same post as Claim #C7 ↑

Insufficient evidence 5

Claim #C10 · Insufficient evidence · Low confidence

Research Quality: Claim: adults with ADHD solve others' problems more easily than their own because others' needs feel urgent, clear and guilt-driven. Everyone reasons more wisely about other people's problems than their own [1], so the pattern is not ADHD-specific; we found no ADHD study of it (GRADE very low). "I asked a therapist" is an anecdote, not a source. An ADHD vs control study of self- vs other-directed task completion would be needed.

X Post Link · Citations for this claim

I ASKED A THERAPIST WHY ADHD ADULTS CAN SAVE EVERYONE ELSE BUT STRUGGLE TO HELP THEMSELVES

1/6 – I ASKED A THERAPIST WHY ADHD ADULTS CAN SAVE EVERYONE ELSE BUT STRUGGLE TO HELP THEMSELVES

(the answer made me realize why I could solve everyone's problems except my own.) 1/6

2/6 – 1. other people's problems feel more urgent.

When someone asks for help, there's a clear task, a person waiting, and often an immediate reason to act. Your own needs can feel less urgent because nobody else is waiting for you to handle them. 2/6

3/6 – 2. helping someone gives you a clear starting point.

You know what needs to be done, what the goal is, and sometimes what the next step should be. With your own problems, everything can feel vague and overwhelming before you even begin. 3/6

4/6 – 3. you may find it easier to organize someone else's life.

Giving advice can feel straightforward when you're looking at a problem from the outside. Applying that same advice to your own routines, emotions, or responsibilities can be much harder when you're already overwhelmed. 4/6

5/6 – 4. guilt can push you to show up for others.

You might respond to someone else's message immediately but postpone your own appointment for weeks. The fear of disappointing another person can create urgency that your personal responsibilities don't seem to generate. 5/6

6/6 – 5. you keep postponing yourself until you have energy left.

You help, listen, solve problems, and meet other people's expectations. Then, when it's finally time to care for yourself, you're mentally exhausted and struggle to start. 6/6

Claim #C11 · Insufficient evidence · Low confidence

Research Quality: Claim: ADHD burnout differs from ordinary exhaustion, doesn't respond to rest, and needs unmasking to recover. Adults with ADHD are more fatigued (62% met fatigue criteria) [1], but "ADHD burnout" has no validated definition; the unmasking-and-recovery idea comes from qualitative autism research [2] (GRADE very low). Red flags: self-report, indirect population, no tests of rest. A validated ADHD burnout measure and a recovery study would be needed.

X Post Link · Citations for this claim

BREAKING: Researchers confirm the number 1 cause of burnout in adults with ADHD isn't workload.

1/6 – BREAKING: Researchers confirm the number 1 cause of burnout in adults with ADHD isn't workload.

It's spending 20 years pretending to be neurotypical.

The masking is more exhausting than any job. 1/6

2/6 – 1. Every interaction has a cost when you are masking. Appear engaged. Do not interrupt. Track the conversation even when your brain is pulling in four directions. Make appropriate eye contact. Remember their name. Respond at the right speed. Not too fast. Not too slow. Every single time. 2/6

3/6 – 2. You do this so consistently and so automatically that eventually you stop noticing you are doing it. The mask becomes the default. And because it is invisible, the cost is invisible. Nobody sees the energy being spent. Including sometimes you. 3/6

4/6 – 3. The burnout arrives when the cost finally exceeds the available resource. It does not come with warning. The person who was managing, who was always fine, who had systems and strategies and could hold it together, suddenly cannot hold it together. And cannot explain why. 4/6

5/6 – 4. ADHD burnout is different from regular exhaustion. It does not respond to rest in a normal way. You can sleep for 10 hours and wake up still depleted. Because the depletion is not in the body. It is in the

system that has been running an invisible parallel process for years. 5/6

6/6 – 5. Recovery from ADHD burnout requires more than rest. It requires removing the mask long enough for the real nervous system to actually recover. That means being somewhere you do not have to perform. With someone who does not require the performance. For long enough to matter. 6/6

Claim #C12 · **Insufficient evidence** · Low confidence

Research Quality: Claim: eating protein every 3–4 hours, "calming" foods (oats, bananas, nuts, yogurt) and slow breaths before meals ease ADHD-related anxiety, and skipped meals can mimic anxiety. Breathing exercises have small effects on stress and anxiety in general adults (g≈−0.3, moderate risk of bias, declared COI) [1], but we found no trials of meal timing or these foods for anxiety in ADHD (GRADE very low). The post's "support, not a cure" hedge is fair. Trials in adults with ADHD and anxiety would be needed.

X Post Link · Citations for this claim

Anxiety and ADHD often show up together, and food can quietly make it better or worse. Here is how to eat in ways that keep your body calmer and your mind steadier. 1/6

Same post as Claim #C1 ↑

Claim #C13 · **Insufficient evidence** · Low confidence

Research Quality: Claim: the phone's "dopamine" hijacks ADHD mornings, and fixes like a phone across the room, a song-per-step playlist as a time cue and a boring fixed breakfast work. None of these tips has been tested in ADHD; breakfast helps same-morning attention in children [1], and environment changes sit inside CBT packages that work [2] (GRADE very low). This repeats the untested-tips pattern of claim 11 (11 Oct). Trials of external time cues in adults with ADHD would be needed.

X Post Link · Citations for this claim

WAKE UP EARLY. JOURNAL. MEDITATE. WHOEVER WROTE THAT MORNING ROUTINE NEVER MET AN ADHD BRAIN.

Same post as Claim #C8 ↑

Claim #C14 · **Insufficient evidence** · Low confidence

Repeat of 11 Oct 2026 Claim #11 – Insufficient evidence (GRADE Very low)
New in this wording: Adds fake deadlines, timed challenges and accountability to body doubling. The classic self-imposed-deadline study was retracted in 2026 [1], body doubling still rests on one small pilot [2], and only CBT packages that include such tactics have been tested [3]. Same verdict.

X Post Link · Citations for this claim

BREAKING: Scientists confirm that ADHD brains physically cannot start a task without dopamine access.

Same post as Claim #C7 ↑

Unsupported 5

Claim #C15 · Unsupported · High confidence

Repeat of 11 Oct 2026 Claim #1 – Unsupported (GRADE Very low)
New in this wording: Now says "the number 1 cause" and "20 years pretending to be neurotypical", and adds that masking becomes invisible to the person. Still no study ranks burnout causes; the only ADHD job-burnout study points to executive-function demands [1]. No new evidence; same verdict.

X Post Link · Citations for this claim

BREAKING: Researchers confirm the number 1 cause of burnout in adults with ADHD isn't workload.

Same post as Claim #C11 ↑

Claim #C16 · Unsupported · High confidence

Research Quality: Claim: scientists confirm ADHD brains can't hold an ongoing felt sense of absent loved ones, causing a distinct loneliness. Young people with ADHD do report more loneliness (meta-analysis, g=0.41, high heterogeneity) [1], but we found no study of this "emotional object permanence" mechanism (GRADE very low), and the long-distance claim is untested. Red flag: "BREAKING: scientists confirm" with no study. Research measuring felt closeness during separation in ADHD would be needed.

X Post Link · Citations for this claim

BREAKING: Scientists confirm that ADHD brains experience loneliness differently.

1/6 – BREAKING: Scientists confirm that ADHD brains experience loneliness differently.

The feeling of someone fades when they're not physically present.

Out of sight doesn't just mean out of mind. 1/6

2/6 – 1. Most people carry a warm ongoing sense of the people they love even when those people are not around. It runs quietly in the background. A kind of ambient emotional presence. ADHD brains have trouble maintaining that ambient state. When you leave, the feeling of you often leaves too. 2/6

3/6 – 2. This creates a specific kind of loneliness that is hard to explain. You can be surrounded by people you genuinely love and feel like none of them are real to you because none of them are physically present right now. The love is not gone. The access to it is. 3/6

4/6 – 3. It also creates confusion for the people in those relationships. They feel the connection when you are together. They feel the absence when you are apart. And because the absence is total and the ADHD person often cannot explain it, it reads as inconsistency or lack of care. 4/6

5/6 – 4. Long distance relationships are particularly hard. Not because the ADHD person cares less. Because the mechanism that allows most people to sustain connection across physical distance is the same mechanism that is impaired in ADHD. The distance is not just miles. It is neurological. 5/6

6/6 – 5. Understanding this does not solve it. But it changes the conversation from "do you even miss me" to "your brain works differently with absence and here is how we build around that." The second conversation is survivable. The first one usually is not. 6/6

Claim #C17 · **Unsupported** · High confidence

Repeat of 11 Oct 2026 Claim #9 – Partially supported (GRADE Low)
New in this wording: Goes further than claim 9: a task "physically cannot start" without dopamine and discipline "cannot work". Dopamine-transporter findings are inconsistent [1], executive deficits are neither necessary nor sufficient for ADHD [2], and effortful CBT skills help [3], so this stronger version is rated Unsupported (GRADE low).

X Post Link · X Post Link 2 · Citations for this claim

BREAKING: Neuroscientists confirm ADHD brains don't have a motivation problem.

Same post as Claim #C7 ↑

ALSO CITED: BREAKING: SCIENTISTS CONFIRM THAT ADHD BRAINS PHYSICALLY CANNOT START A TASK WITHOUT DOPAMINE ACCESS.

Same post as Claim #C7 ↑

Claim #C18 · **Unsupported** · High confidence

Repeat of 11 Oct 2026 Claim #4 – Unsupported (GRADE Low)
New in this wording: Shifts from "shame is the #1 hidden symptom" to "ADHD brains process shame differently", so mistakes are "filed as permanent evidence". No neuroscience study shows a distinct shame-processing system; the related findings are low self-esteem [1] and stronger emotions [2]. Same verdict.

X Post Link · Citations for this claim

BREAKING: Neuroscientists confirm ADHD brains don't process shame the way other brains do.

1/6 – BREAKING: Neuroscientists confirm ADHD brains don't process shame the way other brains do.

One mistake doesn't fade. It gets filed as permanent evidence of who you are. 1/6

2/6 – 1. In a neurotypical brain a mistake gets processed, contextualised, and eventually filed as a past event. It happened. It was bad. It is over. The brain moves forward. In an ADHD brain, particularly one that has been undiagnosed for years, mistakes do not follow that path. 2/6

3/6 – 2. Instead they get added to a running evidence file. The file that has been building since childhood. Every dropped ball, every broken promise, every time you fell short of what was expected. The file does not shrink over time. It compounds. And at some point the file starts to feel like a personality. 3/6

4/6 – 3. This is partly why rejection hits so hard. The rejection is not just this moment. It activates the file. Every previous rejection, every past failure, every piece of evidence that you are fundamentally not enough. A single critical comment does not land as a single critical comment. It lands as confirmation. 4/6

5/6 – 4. It is also why ADHD people are often significantly harder on themselves than any external person is being. The internal critic has access to the full file. It has been making a case for years. It is thorough and it does not take days off. 5/6

6/6 – 5. One mistake is one mistake. That is all it is. The meaning it carries beyond that was added by a nervous system that has been under-resourced and over-criticised for years. The mistake is not evidence. It is data. And data is not a verdict. 6/6

Claim #C19 · **Unsupported** · High confidence

Research Quality: Claim: limerence is ADHD dopamine dysregulation, the ADHD brain is chronically low on dopamine, and this makes people end good relationships. Limerence had no validated measure until 2025 and has no ADHD data [1]; romantic love engages dopamine reward areas in everyone [2], and there is no consistent "low dopamine" finding in ADHD [3] (GRADE very low). Adults with ADHD do have shorter, more troubled relationships [4], but not via this proven route. ADHD studies using the LQ-11 would be a start.

X Post Link · Citations for this claim

WAIT WAIT WAIT so apparently if you become completely obsessed with one person, feel high when they text, devastated when they don't that is not love addiction.

1/6 – WAIT WAIT WAIT so apparently if you become completely obsessed with one person, feel high when they text, devastated when they don't that is not love addiction.

It's called limerence. It's ADHD dopamine dysregulation. And it quietly destroys relationships. 1/6

2/6 – 1. Limerence is an involuntary obsessive state focused on another person. Intrusive thoughts about them. Emotional highs when they respond. Emotional lows when they do not. A constant background hum of that one person that you cannot turn off and cannot fully explain. 2/6

3/6 – 2. The ADHD brain is a limerence risk because it is chronically low on dopamine. A new person with romantic potential floods the brain with more dopamine than it has felt in a long time. The brain does not fall in love. It becomes chemically dependent on the source. 3/6

4/6 – 3. The problem is that limerence feels exactly like love from the inside. It is more intense than most love feels. That intensity is mistaken for depth. For significance. For proof that this is the right person. The intensity is real. But what it is measuring is neurochemistry, not compatibility. 4/6

5/6 – 4. When limerence fades, and it always fades, the ADHD brain often reads the fade as falling out of love. The intensity is gone. The relationship feels flat by comparison. The brain starts looking for the intensity somewhere else. This is how ADHD people end good relationships chasing a feeling that was never sustainable. 5/6

6/6 – 5. Understanding limerence does not make it stop. But it stops you making permanent decisions based on its presence or absence. The fade is not the relationship dying. It is the neurochemistry normalizing. What is left after the fade is the actual relationship. That is the one worth assessing. 6/6

Section 2 · Citations

One group per claim, in the same order as Section 1. Numbers match the [n] markers in each claim's Research Quality note. A source cited by several claims is repeated in each group.

Claim #C1 – Anxiety and ADHD often show up together, and food can quietly make it better or worse. Here is how to eat in ways that keep your body calmer and your mind steadier. 1/6

↑ Back to claim

1. Choi WS, Woo YS, Wang SM, Lim HK, Bahk WM (2022). The prevalence of psychiatric comorbidities in adult ADHD compared with non-ADHD populations: a systematic literature review. *PLOS ONE*. doi.org/10.1371/journal.pone.0277175
   Systematic review 32 studies · No pooled estimate; prevalence varies with diagnostic tool; no COI declared.

Claim #C2 – Anxiety and ADHD often show up together, and food can quietly make it better or worse. Here is how to eat in ways that keep your body calmer and your mind steadier. 1/6

↑ Back to claim

1. Klevebrant L, Frick A (2022). Effects of caffeine on anxiety and panic attacks in patients with panic disorder: a systematic review and meta-analysis. *General Hospital Psychiatry*. doi.org/10.1016/j.genhosppsych.2021.11.005
   Meta-analysis (blinded placebo-controlled challenges) 10 studies; 237 patients, 115 controls · High doses only (400–750 mg, about 5 cups of coffee); only 2 studies reported placebo anxiety data; small samples.
2. Ben Simon E, Rossi A, Harvey AG, Walker MP (2020). Overanxious and underslept. *Nature Human Behaviour*. doi.org/10.1038/s41562-019-0754-8
   Experimental sleep deprivation + population study Lab experiment plus online sample · Healthy adults, not ADHD; an author correction was published (2020) and does not change the main finding.

Claim #C3 – Does when you eat matter as much as what you eat? For ADHD brains, regular meal timing may be one of the simplest levers for steadier focus. Here is what is known, and what is still unclear. 1/7

↑ Back to claim

1. Storebø OJ, Storm MRO, Pereira Ribeiro J, et al. (2023). Methylphenidate for children and adolescents with attention deficit hyperactivity disorder (ADHD). *Cochrane Database of Systematic Reviews*. doi.org/10.1002/14651858.CD009885.pub3
   Cochrane systematic review of RCTs RCTs; 35 trials, 5,342 participants for non-serious adverse events · Names decreased appetite as a typical non-serious adverse event; overall adverse-event certainty rated very low because unblinding is hard to avoid. Updated in 2025 (pub4).

Claim #C4 – Does when you eat matter as much as what you eat? For ADHD brains, regular meal timing may be one of the simplest levers for steadier focus. Here is what is known, and what is still unclear. 1/7

↑ Back to claim

1. Adolphus K, Lawton CL, Champ CL, Dye L (2016). The effects of breakfast and breakfast composition on cognition in children and adolescents: a systematic review. *Advances in Nutrition*. doi.org/10.3945/an.115.010256
   Systematic review of intervention studies 45 studies · Funded by a grant from Kellogg's USA; small samples and insensitive tests; not ADHD-specific; no firm conclusion on breakfast composition.
2. Nazar BP, Bernardes C, Peachey G, Sergeant J, Mattos P, Treasure J (2016). The risk of eating disorders comorbid with attention-deficit/hyperactivity disorder: a systematic review and meta-analysis. *International Journal of Eating Disorders*. doi.org/10.1002/eat.22643
   Meta-analysis (observational) 12 studies; 4,013 ADHD vs 29,404 controls · Pooled OR 3.82 for any eating disorder, binge eating OR 4.13; unexplained heterogeneity; risk higher with interview-based ADHD diagnosis.

Claim #C5 – A psychiatrist explains why your "always running late" habit may sometimes be ADHD-related time blindness.

↑ Back to claim

1. Zheng Q, Wang X, Chiu KY, Shum KK (2022). Time perception deficits in children and adolescents with ADHD: a meta-analysis. *Journal of Attention Disorders*. doi.org/10.1177/1087054720978557
   Meta-analysis 27 studies; 1,620 ADHD vs 1,249 controls · Less accurate (g>0.40) and less precise (g=0.66) timing; moderated by age and gender; lab tasks, children only.
2. Metcalfe KB, McFeaters CD, Voyer D (2024). Time-perception deficits in attention-deficit/hyperactivity disorder: a systematic review and meta-analysis. *Developmental Neuropsychology*. doi.org/10.1080/87565641.2023.2293712
   Meta-analysis (lifespan) 824 effect sizes · Mean g=0.69, moderated by age and working memory; lab timing tasks rather than real-world lateness.
3. Willcutt EG, Doyle AE, Nigg JT, Faraone SV, Pennington BF (2005). Validity of the executive function theory of attention-deficit/hyperactivity disorder: a meta-analytic review. *Biological Psychiatry*. doi.org/10.1016/j.biopsych.2005.02.006
   Meta-analysis 83 studies; 3,734 ADHD vs 2,969 controls · Medium effects (0.46–0.69) on all EF tasks, strongest for inhibition, vigilance, working memory and planning; EF deficits not universal, so neither necessary nor sufficient.

Claim #C6 – 7 SIGNS YOU HAVE HIGH-FUNCTIONING ADHD that nobody in your life has ever noticed.

↑ Back to claim

1. Milioni AL, Chaim TM, Cavallet M, et al. (2017). High IQ may "mask" the diagnosis of ADHD by compensating for deficits in executive functions in treatment-naïve adults with ADHD. *Journal of Attention Disorders*. doi.org/10.1177/1087054714554933
   Case-control (neuropsychological) 51 adults with ADHD, 33 controls · Small sample split by IQ; many uncorrected comparisons; single site; not replicated.
2. Canela C, Buadze A, Dube A, Eich D, Liebrenz M (2017). Skills and compensation strategies in adult ADHD – a qualitative study. *PLOS ONE*. doi.org/10.1371/journal.pone.0184964
   Qualitative interviews 32 outpatients · Describes self-built compensation strategies; no effect sizes; clinic sample; no COI.

Claim #C7 – BREAKING: Neuroscientists confirm ADHD brains don't have a motivation problem.

↑ Back to claim

1. Bunzeck N, Düzel E (2006). Absolute coding of stimulus novelty in the human substantia nigra/VTA. *Neuron*. doi.org/10.1016/j.neuron.2006.06.021
   fMRI experiments (healthy adults) Two fMRI studies · Shows the dopamine midbrain responds to novelty in healthy people; fMRI signal, not measured dopamine release; not ADHD.
2. Arnsten AFT (2009). Stress signalling pathways that impair prefrontal cortex structure and function. *Nature Reviews Neuroscience*. doi.org/10.1038/nrn2648
   Narrative review n/a (mostly animal studies) · Mechanistic review, largely rodent and primate data; NIH-funded; extrapolation to human shame is indirect.

Claim #C8 – WAKE UP EARLY. JOURNAL. MEDITATE. WHOEVER WROTE THAT MORNING ROUTINE NEVER MET AN ADHD BRAIN.

↑ Back to claim

1. Gollwitzer PM, Sheeran P (2006). Implementation intentions and goal achievement: a meta-analysis of effects and processes. *Advances in Experimental Social Psychology*. doi.org/10.1016/S0065-2601(06)38002-1
   Meta-analysis 94 independent tests · Medium-to-large effect of if-then plans (d≈0.65); older synthesis by the method's originators; mostly student and lab samples.
2. Gawrilow C, Gollwitzer PM (2008). Implementation intentions facilitate response inhibition in children with ADHD. *Cognitive Therapy and Research*. doi.org/10.1007/s10608-007-9150-1
   Experimental (lab) Small samples of children · Lab Go/No-Go task, not daily routines; small; co-author is the originator of the technique.
3. Wood W, Rünger D (2016). Psychology of habit. *Annual Review of Psychology*. doi.org/10.1146/annurev-psych-122414-033417
   Narrative review n/a · Authoritative review of habit research; not ADHD-specific.

Claim #C9 – BREAKING: Scientists confirm that ADHD brains physically cannot start a task without dopamine access.

↑ Back to claim

1. Jackson JNS, MacKillop J (2016). Attention-deficit/hyperactivity disorder and monetary delay discounting: a meta-analysis of case-control studies. *Biological Psychiatry: Cognitive Neuroscience and Neuroimaging*. doi.org/10.1016/j.bpsc.2016.01.007
   Meta-analysis (case-control) 25 comparisons, N=3,913 · No publication bias (Egger p=0.53; trim-and-fill d 0.43→0.40); minimal heterogeneity.
2. Zheng Q, Wang X, Chiu KY, Shum KK (2022). Time perception deficits in children and adolescents with ADHD: a meta-analysis. *Journal of Attention Disorders*. doi.org/10.1177/1087054720978557
   Meta-analysis 27 studies; 1,620 ADHD vs 1,249 controls · Less accurate (g>0.40) and less precise (g=0.66) timing; moderated by age and gender; lab tasks, children only.

Claim #C10 – I ASKED A THERAPIST WHY ADHD ADULTS CAN SAVE EVERYONE ELSE BUT STRUGGLE TO HELP THEMSELVES

↑ Back to claim

1. Grossmann I, Kross E (2014). Exploring Solomon's paradox: self-distancing eliminates the self-other asymmetry in wise reasoning about close relationships in younger and older adults. *Psychological Science*. doi.org/10.1177/0956797614535400
   Randomised experiments 3 experiments, N=693 · People reason more wisely about others' problems than their own; general population, not ADHD; not pre-registered.

Claim #C11 – BREAKING: Researchers confirm the number 1 cause of burnout in adults with ADHD isn't workload.

↑ Back to claim

1. Rogers DC, Dittner AJ, Rimes KA, Chalder T (2017). Fatigue in an adult attention deficit hyperactivity disorder population: a trans-diagnostic approach. *British Journal of Clinical Psychology*. doi.org/10.1111/bjc.12119
   Cross-sectional 243 ADHD, 86 CFS, 211 controls · 62% of adults with ADHD met fatigue caseness; self-report; clinic sample; says nothing about rest or masking.
2. Raymaker DM, Teo AR, Steckler NA, et al. (2020). "Having all of your internal resources exhausted beyond measure and being left with no clean-up crew": defining autistic burnout. *Autism in Adulthood*. doi.org/10.1089/aut.2019.0079
   Qualitative (participatory) 19 interviews + 19 internet sources · About autistic, not ADHD, burnout; participants named unmasking and time off as aids to recovery; no quantitative test.

Claim #C12 – Anxiety and ADHD often show up together, and food can quietly make it better or worse. Here is how to eat in ways that keep your body calmer and your mind steadier. 1/6

↑ Back to claim

1. Fincham GW, Strauss C, Montero-Marin J, Cavanagh K (2023). Effect of breathwork on stress and mental health: a meta-analysis of randomised-controlled trials. *Scientific Reports*. doi.org/10.1038/s41598-022-27247-y
   Meta-analysis of RCTs 12 RCTs (N=785) for stress; 20 for anxiety · Small effects (stress g=−0.35, anxiety g=−0.32); most trials at moderate risk of bias; first author trained as a breath teacher (declared).

Claim #C13 – WAKE UP EARLY. JOURNAL. MEDITATE. WHOEVER WROTE THAT MORNING ROUTINE NEVER MET AN ADHD BRAIN.

↑ Back to claim

1. Adolphus K, Lawton CL, Champ CL, Dye L (2016). The effects of breakfast and breakfast composition on cognition in children and adolescents: a systematic review. *Advances in Nutrition*. doi.org/10.3945/an.115.010256
   Systematic review of intervention studies 45 studies · Funded by a grant from Kellogg's USA; small samples and insensitive tests; not ADHD-specific; no firm conclusion on breakfast composition.
2. Liu CI, Hua MH, Lu ML, Goh KK (2023). Effectiveness of cognitive behavioural-based interventions for adults with attention-deficit/hyperactivity disorder extends beyond core symptoms: a meta-analysis of randomized controlled trials. *Psychology and Psychotherapy: Theory, Research and Practice*. doi.org/10.1111/papt.12455
   Meta-analysis of RCTs 28 RCTs · PROSPERO-registered (CRD42021273633); heterogeneous control conditions.

Claim #C14 – BREAKING: Scientists confirm that ADHD brains physically cannot start a task without dopamine access.

↑ Back to claim

1. Ariely D, Wertenbroch K (2002). Procrastination, deadlines, and performance: self-control by precommitment. *Psychological Science*. doi.org/10.1111/1467-9280.00441
Experiments (RETRACTED) Student samples · Retracted by Psychological Science in Sept 2026 (doi:10.1177/09567976261488042); a 2026 replication was published. Listed only to flag it; not used as support.
2. Schuenke R, Dickenson SR, Moore M (2025). Reading between the lines: exploring body doubling in ADHD using EEG. *Proceedings of ACM ASSETS 2025*. doi.org/10.1145/3663547.3759743
Pilot experiment N=26 · Conference paper; small, underpowered; no significant ADHD-specific effect.
3. Liu CI, Hua MH, Lu ML, Goh KK (2023). Effectiveness of cognitive behavioural-based interventions for adults with attention-deficit/hyperactivity disorder extends beyond core symptoms: a meta-analysis of randomized controlled trials. *Psychology and Psychotherapy: Theory, Research and Practice*. doi.org/10.1111/papt.12455
Meta-analysis of RCTs 28 RCTs · PROSPERO-registered (CRD42021273633); heterogeneous control conditions.

Claim #C15 – BREAKING: Researchers confirm the number 1 cause of burnout in adults with ADHD isn't workload.

↑ Back to claim

1. Turjeman-Levi Y, Itzchakov G, Engel-Yeger B (2024). Executive function deficits mediate the relationship between employees' ADHD and job burnout. *AIMS Public Health*. doi.org/10.3934/publichealth.2024015
Cross-sectional field study n=171 · Self-report only; mediation tested on cross-sectional data, so causal direction cannot be established (Newcastle-Ottawa: low).

Claim #C16 – BREAKING: Scientists confirm that ADHD brains experience loneliness differently.

↑ Back to claim

1. Jong A, Odoi CM, Lau J, Hollocks MJ (2024). Loneliness in young people with ADHD: a systematic review and meta-analysis. *Journal of Attention Disorders*. doi.org/10.1177/10870547241229096
Meta-analysis 20 studies (15 meta-analysed) · Higher loneliness in ADHD (g=0.41) with high heterogeneity (I^2=75%); under-25s only; no data on mechanisms; no COI.

Claim #C17 – BREAKING: Neuroscientists confirm ADHD brains don't have a motivation problem.

↑ Back to claim

1. Fusar-Poli P, Rubia K, Rossi G, et al. (2012). Striatal dopamine transporter alterations in ADHD: pathophysiology or adaptation to psychostimulants? A meta-analysis. *American Journal of Psychiatry*. doi.org/10.1176/appi.ajp.2011.11060940
   Meta-analysis 9 PET/SPECT studies, 169 vs 173 · Dopamine-transporter findings inconsistent (high heterogeneity) and linked to prior stimulant exposure, which cautions against a simple 'low dopamine' story.
2. Willcutt EG, Doyle AE, Nigg JT, Faraone SV, Pennington BF (2005). Validity of the executive function theory of attention-deficit/hyperactivity disorder: a meta-analytic review. *Biological Psychiatry*. doi.org/10.1016/j.biopsych.2005.02.006
   Meta-analysis 83 studies; 3,734 ADHD vs 2,969 controls · Medium effects (0.46–0.69) on all EF tasks, strongest for inhibition, vigilance, working memory and planning; EF deficits not universal, so neither necessary nor sufficient.
3. Liu CI, Hua MH, Lu ML, Goh KK (2023). Effectiveness of cognitive behavioural-based interventions for adults with attention-deficit/hyperactivity disorder extends beyond core symptoms: a meta-analysis of randomized controlled trials. *Psychology and Psychotherapy: Theory, Research and Practice*. doi.org/10.1111/papt.12455
   Meta-analysis of RCTs 28 RCTs · PROSPERO-registered (CRD42021273633); heterogeneous control conditions.

Claim #C18 – BREAKING: Neuroscientists confirm ADHD brains don't process shame the way other brains do.

↑ Back to claim

1. Pedersen AB, Edvardsen BV, Messina SM, et al. (2024). Self-esteem in adults with ADHD using the Rosenberg Self-Esteem Scale: a systematic review. *Journal of Attention Disorders*. doi.org/10.1177/10870547241237245
   Systematic review 11 studies · Cross-sectional studies, little confounder control; no pooled estimate; no COI.
2. Beheshti A, Chavanon ML, Christiansen H (2020). Emotion dysregulation in adults with attention deficit hyperactivity disorder: a meta-analysis. *BMC Psychiatry*. doi.org/10.1186/s12888-020-2442-7
   Meta-analysis 13 studies, N=2,535 · PROSPERO-registered; authors report an asymmetric funnel plot (possible publication bias / small-study effects), so g=1.17 may be inflated; no COI.

Claim #C19 – WAIT WAIT WAIT so apparently if you become completely obsessed with one person, feel high when they text, devastated when they don't that is not love addiction.

↑ Back to claim

1. Marshall L, Waldeck D, Pancani L, Churchill S, Tyndall I (2025). Development and validation of the Limerence Questionnaire (LQ-11). *Psychological Reports*. doi.org/10.1177/00332941251394980
   Scale development (cross-sectional) n=269 and n=401 · States that no published limerence measure existed before; no ADHD or dopamine data.
2. Fisher H, Aron A, Brown LL (2005). Romantic love: an fMRI study of a neural mechanism for mate choice. *Journal of Comparative Neurology*. doi.org/10.1002/cne.20772
   fMRI (healthy adults) Small sample of people newly in love · Romantic love engages dopamine-rich reward regions in healthy people; small, not ADHD, fMRI signal not dopamine levels.
3. Fusar-Poli P, Rubia K, Rossi G, et al. (2012). Striatal dopamine transporter alterations in ADHD: pathophysiology or adaptation to psychostimulants? A meta-analysis. *American Journal of Psychiatry*. doi.org/10.1176/appi.ajp.2011.11060940
   Meta-analysis 9 PET/SPECT studies, 169 vs 173 · Dopamine-transporter findings inconsistent (high heterogeneity) and linked to prior stimulant exposure, which cautions against a simple 'low dopamine' story.
4. Wymbs BT, Canu WH, Sacchetti GM, Ranson LM (2021). Adult ADHD and romantic relationships: what we know and what we can do to help. *Journal of Marital and Family Therapy*. doi.org/10.1111/jmft.12475
   Narrative review n/a · Finds adults with ADHD tend to have shorter, more discordant relationships; notes few studies on why.

About this report

19 claims from 12 threads, checked with the Scientific Evidence Evaluator framework. The practical, hedged threads (meal timing, anxiety and food, time blindness) held up best. Every "BREAKING: scientists/neuroscientists confirm" headline failed again: none named a study, and five repeat or overstate claims already rated on 11 Oct (C15 = claim 1, C18 = claim 4, C7 and C17 = claim 9, C14 = claim 11). These are shown as short repeat cards that link to the earlier verdict, with a note on what is new in the wording. Note: the classic self-imposed-deadline study (Ariely & Wertenbroch 2002) was retracted in Sept 2026, so deadline tips lose that support.

How claims were evaluated. Evidence hierarchy: Systematic reviews and meta-analyses (Level 1), RCTs (2), cohort (3), case-control (4), cross-sectional (5), case reports and expert opinion (6). Guidelines and clinical reviews are named as such. **GRADE certainty:** Our rating of the body of evidence for this specific claim: randomised evidence starts High and observational starts Low. Ratings go down for risk of bias, inconsistency, indirectness (e.g. non-ADHD or lab-only samples), imprecision and publication bias, and up for large effects. **Verdicts:** Supported: strong, consistent, high-quality evidence. Partially supported: some solid evidence, but the claim overgeneralises or has important caveats. Insufficient evidence: too little good-quality research to judge. Unsupported: evidence is absent for the claim as stated, or contradicts it (flagged as contradicted). Confidence is our confidence in the verdict, which can be high even when GRADE is low (e.g. a well-hedged claim).

Verdict changes: None. No new part extended a thread checked earlier, so no earlier verdict was re-assessed. Repeated claims point back to the 11 Oct verdicts, which still stand.

Method. Posts were pulled with the X API. Self-reply parts were reconstructed via conversation_id and reply links and quoted verbatim, numbered in order. Reference metadata, corrections and retractions were checked against Crossref and PubMed, and open-access full texts were read. GRADE ratings are our own per-claim judgement unless stated.

Not medical advice. Verdicts reflect evidence as of the check date and are group-level. Talk to a qualified clinician about diagnosis or treatment.